

Prescribing Skills 1 and Advanced Clinical Skills 1 OSCE Rubric for stations

All stations are must-pass components

Criteria	HD	D	C	P	F	Points	Station
Patient history and Information gathering Obtaining relevant clinical information from patients	15 to >13.0 Pts All relevant patient information collected: conducts a focused, patient interview using a structured sequence; prioritises key discriminators early; clarifies ambiguity with timely probing; uses brief summaries/signposting to verify accuracy without derailing flow; obtains medicines history, allergies/ADRs, relevant comorbidities and contextual factors; identifies red-flag cues without prompting // All relevant information about patient request/need collected, including goals, expectations and constraints that influence safe care.	13 to >11 Pts Most relevant patient information collected: interview is mostly structured and targeted; explores most key discriminators and medication risks; uses occasional clarification/summaries to confirm information; minor omissions that do not materially alter immediate safety or next steps // Most relevant information about patient request/need collected, with minor gaps in context, priorities or constraints.	11 to >9 Pts Some relevant patient information collected: interview gathers a reasonable dataset but structure/prioritisation is inconsistent; switches between topics or asks questions out of sequence; explores some discriminators and medication risks but omits important elements or obtains them late; clarification/summaries are limited or inconsistent // Some relevant information about patient request/need collected but context and priorities are only partially established.	9 to >7.5 Pts Minimum relevant patient information collected: interview is largely checklist-style or unfocused; relies on prompts to identify important areas; misses multiple key discriminators or medication-risk elements; limited clarification of the presenting request/need; little/no summarising to confirm key facts // Minimum relevant information about patient request/need collected with limited exploration of expectations, preferences or constraints.	7.5 to >0 Pts Insufficient or non-relevant patient information collected: interview is incomplete, inaccurate or poorly directed; fails to elicit key discriminators and/or medication risks; does not establish the patient's request/need in a usable way for safe decisions; information collected is unreliable due to lack of clarification/verification when uncertainty arises.	15	1
Communication Skills and Cultural Safety Features of communication: Empathy, Active listening. Verbal/non-verbal. Use of language. Interview structure and flow. Teachback, Cultural safety elements	20 to >17.0 Pts Always understands and respect patients beliefs and cultural values during encounter // Exemplary interviewing technique // Approached patient in a professional manner // Efficient, appropriate and effective questioning // Optimal use of communication skills in engagement of patient	17 to >15.0 Pts Mostly understands and respect patients beliefs and cultural values during encounter // Proficient interviewing technique // Approached patient in a professional manner // Appropriate and effective questioning // Clear use of communication skills in engagement of patient	15 to >13.0 Pts Mostly understands and respect patients beliefs and cultural values during encounter // Capable interviewing technique // Approached patient in a professional manner // Adequate questioning // Use of some communication skills in engagement of patient	13 to >10.0 Pts Limited understanding and respect for patients beliefs and cultural values during encounter // Adequate interviewing technique // Approached patient in a professional manner // Rudimentary but sufficient questioning // Use of limited communication skills in engagement of patient	10 to >0 Pts No understanding and respect for patients beliefs and cultural values during encounter // Poor interviewing technique // Approached patient in an unprofessional manner // Inadequate, inappropriate or insufficient questioning // No use or incorrect use of communication skills with no patient engagement	20	1
Physical Assessment Delivering an appropriate patient physical assessment for clinical signs and symptoms	15 to >13.0 Pts Performs the specified FPE systematically and aseptically where relevant; positions the patient appropriately and uses correct technique throughout // Obtains accurate vitals efficiently (or correctly recognises provided vitals) // Actively identifies key cues/findings relevant to the specified FPE and responds accurately to all standard examiner interpretation questions, explicitly linking findings to clinical significance (e.g., severity, red flags, likely next steps) // Always maintains patient comfort, dignity and safety.	13 to >11 Pts Performs the specified FPE appropriately with minor non-critical omissions or minor technique lapses; sequence remains mostly systematic // Obtains accurate vitals without difficulty (or correctly uses provided vitals) // Identifies most key cues/findings and answers most standard interpretation questions accurately; links findings to clinical significance with minor gaps // Maintains patient comfort and safety.	11 to >9 Pts Performs the specified FPE adequately but with inconsistent sequencing or multiple minor omissions; technique is acceptable but not consistently confident // Obtains vitals with some difficulty but accuracy is acceptable (or uses provided vitals with minor errors/omissions) // Identifies some cues/findings; answers some interpretation questions correctly but explanations are generic or incomplete // Maintains patient comfort and safety.	9 to >7.5 Pts Performs a basic/minimum version of the specified FPE with multiple omissions, inefficient sequence, or notable technique issues; may require prompting to complete key components // Struggles to obtain vitals (borderline accuracy) or misuses provided vitals // Limited recognition of cues/findings; answers few interpretation questions correctly and provides superficial reasoning // Maintains patient safety but comfort/dignity may be inconsistently addressed.	7.5 to >0 Pts Inadequate or unsafe performance of the specified FPE with major omissions or incorrect technique // Vitals not obtained, clearly inaccurate, or misinterpreted/misused // Key cues/findings not recognised or ignored; interpretation questions answered incorrectly in a way that would lead to unsafe decisions // Risk of patient harm and/or failure to maintain safety.	15	2 and 3
Diagnosis Developing a differential diagnosis process and dealing with uncertainty. Communicating the findings. Answering queries	15 to >13.0 Pts Performs a differential diagnosis (≥2–3 plausible options) and explicitly links key supporting and refuting findings to each option; reaches a justified working diagnosis OR a justified uncertainty with clear rationale (What is known/unknown and why) // Explains the diagnosis or uncertainty in an exemplary, patient-centred manner, including implications and next steps // Correctly answers all queries during the examination.	13 to >11 Pts Performs a differential diagnosis (≥2–3 plausible options) with reasonable prioritisation; provides justification by linking most key findings to the leading option(s) and acknowledges relevant uncertainty // Explains the diagnosis or uncertainty appropriately and clearly // Correctly answers most queries during the examination.	11 to >9 Pts Performs a differential diagnosis (≥2–3 options) but prioritisation is limited or inconsistent; justification relies on partial linkage to findings and may omit key refuting features or gaps // Explains the diagnosis or uncertainty adequately but with limited clarity on reasoning // Correctly answers some queries during the examination.	9 to >7.5 Pts Performs a limited differential diagnosis (≥2 options) with minimal prioritisation; justification is superficial (lists options without clearly connecting to findings) and uncertainty is stated without clear rationale // Explains the diagnosis or uncertainty in a rudimentary but broadly accurate manner // Answers some queries during the examination.	7.5 to >0 Pts No differential diagnosis OR differential diagnosis is inappropriate // Inaccurate diagnosis or unjustified uncertainty (cannot explain why) // No or incorrect explanation of diagnosis/uncertainty // No answers to queries or incorrect answers during the examination.	15	4
Management and follow up plan Pharmacological and non-pharmacological recommendations, and/or referral based on the diagnosis or uncertainty identified	30 to >28.0 Pts Optimal management given: optimal product/therapy selection, dose, route (if relevant) AND/OR Referral when required; instructions explicitly aligned to the working diagnosis or justified uncertainty; checks understanding // Optimal non-pharmacological management provided: comprehensive, practical advice tailored to the presentation; checks understanding // Optimal safety-netting and follow-up provided: explicit timeframe for review and clear triggers for escalation/cessation/urgent care.	28 to >24.0 Pts Appropriate management given: appropriate product/therapy selection, dose AND/OR Referral when required; instructions aligned to the working diagnosis or uncertainty; checks understanding // Appropriate non-pharmacological management provided: comprehensive advice with minor omissions; checks understanding // Appropriate safety-netting and follow-up provided: review timeframe stated and most key escalation triggers included.	24 to >20.0 Pts Adequate management given: adequate product/therapy selection AND/OR Referral when required; instructions broadly appropriate but may be generic or incompletely matched to the diagnosis/uncertainty; checks understanding (may be limited) // Adequate non-pharmacological management provided: basic advice with omissions; checks understanding (may be limited) // Adequate safety-netting and follow-up provided: follow-up plan present but lacks specificity (timeframe and/or triggers incomplete).	20 to >15.0 Pts Adequate but minimal management given: adequate product/therapy selection AND/OR Referral when required, but instructions are incomplete or not clearly aligned to diagnosis/uncertainty // Adequate but minimal non-pharmacological advice provided: // Limited safety-netting and follow-up: follow-up mentioned but timeframe and triggers are vague or largely absent.	15 to >0 Pts Inappropriate management based on treatment/referral requirements OR inappropriate advice // Harm or risk of harm caused to the patient // No safety-netting and follow-up provided.	30	4
Clinical procedure Applying correctly a clinical procedure as deemed necessary during the treatment plan	15 to >13.0 Pts Obtains consent (confirms understanding and checks allergies/concerns); performs safety checks appropriate to the procedure (right procedure/product, dose/volume if relevant, route, site, expiry/back integrity, and contraindications as applicable) // Exemplary aseptic technique maintained (no contamination events; appropriate hand hygiene/PPE and field management) // Preparation and execution are accurate, efficient and controlled; anticipates and prevents common errors // Effective pain management and patient comfort measures applied appropriately // Provides complete post-procedure instructions and ensures safe disposal.	13 to >11 Pts Obtains consent and performs safety checks with minor non-critical gaps (does not compromise overall safety) // Correct aseptic technique with no major breaks; minor lapses corrected promptly // Preparation and execution are correct with minor inefficiencies // Pain management and comfort considered appropriately // Post-procedure instructions and disposal are correct.	11 to >9 Pts Obtains consent but explanation/verification is limited OR performs safety checks with some omissions that require correction // Aseptic technique is generally correct but with occasional lapses or uncertainty; maintains a mostly safe field // Preparation and execution are broadly correct but include technique gaps or hesitations // Pain management/comfort measures are basic or inconsistently applied // Post-procedure instructions/disposal are provided but incomplete.	9 to >7.5 Pts Consent obtained but incomplete (limited verification) OR safety checks performed superficially and require prompting to address key items // Aseptic technique is attempted but with major difficulties (risk of contamination) // Preparation/execution show technique problems or poor control // Pain management/comfort measures are limited or inappropriate // Post-procedure instructions/disposal are limited.	7.5 to >0 Pts Consent not obtained/ignored OR safety checks not performed or have major gaps // Incorrect or unsafe aseptic technique (contamination not recognised or corrected) // Incorrect preparation or execution // Incorrect or no pain management if required // Incorrect or lack of post-procedure instructions or disposal // Incorrect or no documentation of lot/expiry where applicable.	15	5 and 6
Documentation Follow legal and ethical obligation by documenting the clinical encounter for communicating findings to the care team	10 to >9 Pts Clear, concise, complete SOAP communication/referral to GP or urgent care/ED as appropriate // States patient consent to share // Includes key S/O data, A (diagnosis/uncertainty + risks), and P (what was done + explicit request + urgency/destination + meds + follow-up/safety-netting) // Accurate terminology // Timed and signed, legally/professionally compliant.	8 to >7 Pts SOAP communication/referral is complete and mostly well prioritised // Consent recorded // Appropriate recipient/urgency stated // Plan includes explicit request and key management details with minor omissions // Timed and signed, professional/legal compliant.	7 to >6 Pts SOAP present but prioritisation/clarity inconsistent // Consent mentioned but brief/unclear // Recipient/urgency or request not fully explicit // Some key details missing (e.g., objective data, rationale, meds, follow-up/safety-netting) // Timed and signed, professional/legal compliant.	6 to >5 Pts Limited/partial SOAP // Consent ambiguous // Recipient/urgency and/or explicit request unclear // Important details omitted // Minimally compliant (signature/time may be incomplete).	5 to >0 Pts Unsafe or incomplete communication: SOAP not functional and/or major errors/omissions // Consent not recorded // Urgency/destination or plan unsafe/unclear // Not signed/timed or not legally/professionally compliant.	10	7
TOTAL						120	

* Students failing any criteria will need to repeat all components related to that criteria (colour coded)